Elevator ER

By Jon Levine

When I was young, I was a bright, happy, enthusiastic kid. Years later I had become and angry, sullen, disaffected high school dropout.

If I could've signed a form to officially drop out of American society or the human race, that would've been next. So when I applied for a job working the night shift in the OR, it was not out of a great desire to help my fellow man. I was only doing it because the job paid really well -- $9 an hour.

The fact that the hospital hired me at that time in my life should be sufficient to scare all of you into taking excellent care of yourselves from now on. But hire me they did, and my actual job title was OR nursing assistant, night shift.

INTRODUCTION

Jon Levine was lost . . . floating through life. No goals, no purpose, no ambition. But all that changed one fateful night in the most unexpected way possible.

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The most fundamental part of the job was transporting patients to surgery, which I was trained to do during the day shift when the patients are in stable condition, usually awake but mildly sedated, and it's pretty simple.
On the night shift **nothing** is scheduled. Patients are usually kinda bloody. Sometimes they're highly agitated and need to be restrained. But for the most part they were completely unconscious. I didn't see them as human beings at a time of great need so much as packages that needed to be delivered to a specific room as efficiently as possible.

You know:

- "Burst appendix to OR 22,"
- "Multiple stab wounds to OR 27,"
- "Guy who shot his face off to OR 33."

All manner of human suffering, but to me, it was all the same.

So, it's three in the morning, and there's a call from the ICU. They have a patient. So I go up to get him, and the nurses are disconnecting him from his respirator and his EKG and attaching a portable heart monitor to the rolling ICU bed and an air bag to his breathing tube, which I'm going to

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have to squeeze to breathe for him during the trip. This means that someone's gonna have to come with me, because you can't really steer an ICU bed and squeeze an air bag at the same time.

Unfortunately, the nurses in the ICU couldn't spare anyone except a young woman named Melissa, a nursing student. She was very nervous because she'd never been anywhere else in the hospital before and had never really been given much responsibility. But the nurses assured us that, despite appearances, Mr. Williams was pretty much okay and this would be totally routine.

So I'm pushing from the back and squeezing the air bag with my free hand, and I have Melissa steering from the front as I direct her on the shortest route to the OR**.**

So this old, crappy elevator arrives. We get in. The doors close. I push the button. And nothing happens. And then the lights go out. We start to move. But it's not right, and we're moving too fast, and the elevator's not making its normal sound. And my stomach is in my

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throat, and we're *falling,* **WE'RE FALLING!**

And if I had had time for a thought process, it probably would've been like, ***I'm not ready to die. I didn't sign up for this.*** But before any of that can take place, the elevator's emergency brake kicks in and slams us to a stop so violently that I'm thrown to my knees and Mr. Williams is bouncing in his bed.

So now we're stopped in this tiny, dark box, and there's three sounds I can hear: the elevator's emergency signal buzzing, Melissa screaming, and Mr. Williams's heart monitor indicating that, like our elevator, his heart has stopped.

No, no, no, no, NO! But denial and anger quickly give way to, in this case, bargaining: *Well, Melissa's got some actual medical education, so she should be in charge.*

But then reality sets in. *Melissa's cowering in the corner, wondering if we're all gonna die. But if we don't do something right away,* ***ONE*** *of us is* ***DEFINITELY*** *gonna die.*

So I then hit acceptance and admitted to myself that I

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had to get in the game here. Fortunately, the hospital had trained me to do CPR when they hired me. So I had Melissa stand up, and by the dim lights of the LEDs on the heart monitor, I said, "Okay, you do the air bagging. I'm going to do chest compressions."

I moved around to the side, but Mr. Williams was a big guy, and I couldn't get good leverage. So I did something I had seen one of the emergency ward doctors do one time. I climbed up, sort of mounting Mr. Williams, and did chest compressions from above. And it seemed to work.

I have no idea how long we were like that in the dark, but it felt like hours, until the lights flickered and came back on, and the buzzing emergency thing stopped. We finally arrived at the third-floor OR and the doors opened. And Melissa was so eager to get us out of there, she started pushing us forward. And that's when something ***bad*** happened.

The wheels on the front of the ICU bed swiveled around ninety degrees, right as they were situated over the gap between the elevator and the floor and slid down into the gap

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like slices of bread going into a toaster.

And then, of course, the elevator doors started slamming on us over and over again. I was still doing chest compressions, and we both started screaming for help. Finally, someone showed up, a wonderful older gentleman named Mr. Selwin, who was one of the night cleaning crew.

He comes running in, and in his thick island accent says, "What's all the commotion? Oh my God! Keep doing what you're doing. I'll go get more help." And he runs out.

Within a few moments, he comes back with "reinforcements" and finally, with a horrible metal scraping sound, they wrench us free. And suddenly all the panic and confusion recede, and we are in motion. We're off to OR 29!

The double doors open, and we're whipping around corners and everything's very effortless, and we're flying now - Melissa is running, trying to keep up, while still doing the air bagging.

And of course, I'm still on top of Mr. Williams, still pumping

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away at his chest, and there's a breeze in my face from our newfound velocity. And I have to say, it was absolutely exhilarating, to the point where I had to stifle the urge to laugh out loud.

So I screw my face up into a mask of seriousness because it would be a tad unseemly to appear to be enjoying riding a patient down the hall. And we get to OR 29, and the entire surgical team fly into action and seamlessly take over for everybody. They save him, and he lives. Not only did he live through the night, but he recovered and left the hospital!

That exhilarating feeling didn't go away. It seems kind of obvious in retrospect, but at the time I was so unaccustomed to feeling anything positive, that it took me a really long time to recognize that I enjoyed what I did, and I enjoyed caring about it. I enjoyed caring about Mr. Williams and myself.

Six months later, I found myself mentioning the incident in a personal essay that was part of an application to get back into college.

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AWARD HISTORY

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